

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<reasoning process here> The child presents with symptoms of inattention, impulsivity, hyperactivity, poor concentration, and disruptive behavior, which are classic signs of attention-deficit/hyperactivity disorder (ADHD). The history of being expelled for hitting and being disruptive, along with the difficulty in focusing and listening, supports this diagnosis. The fact that he meets all developmental milestones and has no neurological or physical abnormalities rules out other organic causes. Although the mother has major depressive disorder and the father has Graves' disease, these are not directly linked to the current presentation. There is no indication of hearing loss or sensory impairment that would necessitate hearing aids. While behavior therapy is a first-line treatment for ADHD, especially in young children, stimulant medications like methylphenidate are often considered first-line in clinical practice. However, among the listed options, fluoxetine (a selective serotonin reuptake inhibitor) is not typically first-line for ADHD in young children, though it may be used in certain cases. Behavior therapy is a non-pharmacological, evidence-based first-line intervention, especially in preschool-aged children, and is often recommended before pharmacologic therapy. Given the lack of urgency or red flags for organic causes, and the fact that the child is not yet in a situation requiring medication, the most appropriate next step in a child with suspected ADHD is behavioral intervention. Therefore, behavior therapy is the most appropriate initial step in treatment. Fluoxetine (C) is not first-line for ADHD, and methimazole (B) is used for hyperthyroidism, which is not indicated here. Hearing aids (D) are not indicated. Thus, the best next step is behavior therapy.

</reasoning process>

<answer>A</answer>